Real Case Walkthroughs

From ER and Oncology: What the Experienced Nurse Actually Does

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Case 1: "Chest Pain, Rule Out MI" — The Case That Almost Got Away

The Presentation

3:47 AM. I'm the charge nurse in a 20-bed ER. The triage nurse calls out:

"Nnamdi, I need you to see this one."

Patient: 54-year-old male, brought in by wife. Complains of "indigestion" for 2 hours. Took Tums, didn't help.

First Impressions (Recognize Cues)

Walking into the room, I see:

- Patient is diaphoretic (sweating profusely)
- He's clutching his chest, not his epigastrium
- Wife says he's "never sick, never complains"
- Skin is pale, cool

My immediate thought: This is NOT indigestion.

Vital Signs

Vital	Value	My Interpretation
BP	168/94	Hypertensive response to pain/stress
HR	102	Tachycardic — cardiac or pain response
RR	22	Increased work of breathing
SpO2	95% room air	Slightly low, apply O2
Temp	98.4	Afebrile

My Actions (In Order)

Step 1 (0 seconds): Apply O2 2L NC, continuous monitoring **Step 2** (30 seconds): 12-lead EKG **Step 3** (2 minutes): IV access x2, draw labs (troponin, CBC, BMP, PT/INR) **Step 4** (3 minutes): Aspirin 325mg chewable (if not allergic) **Step 5** (5 minutes): Nitroglycerin 0.4mg SL x1 **Step 6** (5 minutes): Notify MD — "I have a possible STEMI, EKG pending"

The EKG

EKG shows: ST elevation in leads II, III, aVF.

My interpretation: Inferior wall MI.

Critical consideration: Inferior MI can involve the right ventricle. If it does, giving nitroglycerin can cause catastrophic hypotension.

Action: Obtain right-sided EKG (V4R). Hold further nitro until RV involvement ruled out.

What Happened Next

- Right-sided EKG: No RV involvement
- Troponin: 2.4 (elevated, trended to 8.6 at 6 hours)
- Cardiology activated
- Patient went to cath lab within 45 minutes
- Door-to-balloon time: 62 minutes

What I Almost Missed

The patient kept saying "it's just indigestion." He almost convinced me to slow down. But his diaphoresis + clutching chest + "never complains" history = cardiac until proven otherwise.

Lesson: Trust your assessment over the patient's self-diagnosis. They don't know what MI feels like. You do (or you're learning).

NGN-Style Questions This Case Tests

1. **Priority**: What's your FIRST action?
    - Answer: Apply O2 and get EKG (can do simultaneously)
2. **Analysis**: Why hold nitroglycerin?
    - Answer: Possible RV involvement → nitro causes hypotension
3. **Delegation**: What can the tech do while you assess?
    - Answer: Apply O2, obtain vitals, prepare EKG machine
4. **Evaluation**: How do you know your interventions worked?
    - Answer: Pain relief, ST resolution, stable vitals

Case 2: Neutropenic Fever — The Quiet Emergency

The Presentation

Oncology unit. I'm covering 6 patients. The tech calls me:

"Room 14's temp is 100.6."

Patient: 67-year-old female, AML, Day 10 post-chemo. ANC is 400 (severely neutropenic).

My Immediate Response

This is an emergency. Neutropenic fever is one of the few situations where a temperature of 100.6 is more dangerous than 103°F in a healthy person.

Actions (Time-Critical)

Minute 0: Reassess temp myself. Verify with different thermometer. **Minute 1**: Full assessment — any other symptoms? Cough? Dysuria? Skin changes? Mouth sores? **Minute 2**: Blood cultures x2 (from different sites), BEFORE antibiotics **Minute 3**: Urinalysis and culture **Minute 4**: Call MD — "Neutropenic fever, cultures obtained, need antibiotic orders" **Minute 5**: Broad-spectrum antibiotics per protocol (typically cefepime or piperacillin-tazobactam) **Minute 10**: Consider chest X-ray if respiratory symptoms

Why This Is An Emergency

Neutropenic patients have no immune system. An infection that would cause a mild fever in a healthy person can become septic shock in hours.

The standard: Broad-spectrum antibiotics within 60 minutes of fever recognition.

What the Textbook Doesn't Teach You

The family: They see 100.6 and think "low-grade fever, not a big deal." You have to explain — quickly and clearly — why this IS a big deal without causing panic.

What I say: "Because of the chemotherapy, her body can't fight infections right now. A small infection can become serious very quickly. We're starting strong antibiotics right away as a precaution. We're being proactive, not because something is seriously wrong, but because we need to prevent it from becoming serious."

Common Mistake New Grads Make

Waiting for the MD to assess before starting antibiotics. **Don't wait.** Get cultures, start antibiotics per protocol, notify MD. This is one of the few times you act first and notify second.

NGN-Style Questions

1. **Priority**: What's the FIRST nursing action?
    - Answer: Obtain blood cultures, then start antibiotics
2. **Analysis**: Why cultures before antibiotics?
    - Answer: Need to identify organism for targeted therapy
3. **Delegation**: What can the tech do?
    - Answer: Obtain vitals, help with cultures, prepare antibiotics

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Case 3: Post-Op Complications — When "Stable" Changes

The Presentation

Med-surg unit. My patient is post-op laparoscopic cholecystectomy, day 2. Has been stable all day.

4:00 PM assessment:

- BP 118/72 (baseline was 132/78 this morning)
- HR 104 (was 78 this morning)
- RR 20

- SpO2 96% room air
- Pain 4/10
- Says she feels "a little dizzy"

The Change

Parameter	This AM	Now	Change
BP	132/78	118/72	↓ 14/6
HR	78	104	↑ 26
Pain	2/10	4/10	↑ 2

HR up + BP down + increased pain = possible internal bleeding

My Actions

Step 1: Reassess abdomen — distended? Guarding? Rebound? **Step 2**: Check dressing — any drainage? Increased saturation? **Step 3**: Check H&H — compare to post-op baseline **Step 4**: Trend vitals — is this getting worse? **Step 5**: Call MD — "Possible post-op bleed, vitals changed, need H&H and assessment"

What Happened

H&H dropped from 11.2/33 to 8.4/26. MD ordered CT abdomen. Found 400mL hemoperitoneum. Patient went back to OR for evacuation.

The Lesson

Trends matter more than single values. A BP of 118/72 is normal. But it's NOT normal for this patient who was 132/78 this morning. The combination of changes — HR up, BP down, pain up — is the signal. Any ONE of these alone might not be concerning. Together, they're a pattern.

NGN-Style Questions

1. **Recognize**: What cues are concerning?
   - Answer: Trending BP down, HR up, pain increased
2. **Analyze**: What's the most likely cause?
   - Answer: Post-operative bleeding
3. **Prioritize**: What's your first action?
   - Answer: Full assessment + notify MD + obtain H&H

Case 4: The Patient Who Fell — Legal and Clinical

The Presentation

3:00 AM. Call light from Room 8. Patient on the floor next to bed.

Patient: 79-year-old male, admitted for syncope workup. Fall risk bracelet on. Bed alarm was on but silenced earlier by family.

Immediate Actions

Step 1: Assess patient — conscious? Responsive? Pain? Obvious injuries? **Step 2**: Call for help — you need another nurse/tech to assist **Step 3**: Do NOT move patient until assessed — possible fracture/spinal injury **Step 4**: Vital signs — especially neuro checks **Step 5**: Notify MD **Step 6**: Notify charge nurse/house supervisor

The Documentation (Critical)

Falls require EXACT documentation:

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0347: Found patient on floor next to bed. Bed alarm was off.
Patient states "I needed to use the bathroom."
Alert and oriented x3. Complains of mild right hip pain.
No obvious deformity. Full neuro assessment WNL.
Vitals: BP 142/84, HR 88, RR 18, SpO2 95% RA.
MD notified. X-ray ordered. Family notified.
Bed alarm reactivated. Call light within reach.
Non-skid socks applied. Teach reinforcement provided.
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What New Grads Miss

1. **Not documenting the exact time** — falls are time-sensitive legally
2. **Not assessing neuro status** — head injury can be delayed
3. **Not notifying family** — they need to hear it from you first
4. **Not reassessing frequently** — neuro checks q2h for 24 hours minimum

Legal Protection

Your documentation is your license protection. Be objective, factual, and thorough. Never document "patient fell because..." (blame). Document what you found and what you did.

Case 5: First Code Blue — What I Wish Someone Had Told Me

The Scenario

6 months into my first nursing job. I'm in a patient's room giving meds. Overhead:

"Code Blue, Room 312. Code Blue, Room 312."

Room 312 is three doors down. My patient is stable. I run.

What I Found

- Patient unresponsive in bed
- Family in hallway, screaming
- One nurse already doing compressions
- Another nurse grabbing the crash cart

What I Did (As a New Grad)

I asked: "What do you need me to do?"

The charge nurse said: "Get the family out and start documenting."

My Role: Documenter

I grabbed the code record and started writing:

- Time code called
- Time compressions started
- Who was doing compressions
- Medications given (time, dose, route)
- Rhythm strips
- Defibrillation (if any) — joules, time
- Outcome

What I Learned

1. **Codes are chaotic** — Even experienced teams are chaos. Your job is to find a role and do it.

2. **The documenter role is CRITICAL** — Legal protection, quality review, learning opportunity. Take it seriously.

3. **Family needs someone** — Someone should be with them, explaining. Often social work or chaplain. If not you, delegate.

4. **Debrief after** — Every code needs a debrief. What went well? What could improve? This is how you learn.

5. **Self-care** — You'll shake after. You'll cry in the bathroom. That's normal. Talk to someone.

NGN-Style Questions

1. **Priority**: What's your role as the new grad who arrives first?

   - Answer: Assess scene, start compressions if no pulse, call for help

2. **Delegation**: What can family do?

   - Answer: Nothing during the code. Move them out.

3. **Evaluation**: How do you know compressions are effective?

   - Answer: $ETCO_2$ >10, palpable pulse during pauses, rhythm change

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These cases are based on real experiences (de-identified). The thinking patterns are what the NGN NCLEX tests. The complete system includes 25+ more cases with full walkthroughs and video explanations.